

Ankle Fractures

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Basic Anatomy

The three main bones within the ankle joint are: the Tibia, the Fibula and the Talus. These bones are connected by ligaments, muscles, and tendons.

What is an Ankle Fracture?

A fracture is the same as a break. There are different types of ankle fracture depending on which bone and where abouts it is fractured.

How is it Treated?

Most fractures will heal by themselves but do need a period of protected immobilisation in a cast to allow this healing to occur. This will normally take approximately 6 weeks, but this does vary depending on the individual and the type of fracture.

It is important to keep your leg elevated when at rest to reduce swelling. You will be followed up in fracture clinic and may be referred to Physiotherapy Outpatients for on-going management.

Types of ankle casts

Backslab

This is a non-weight bearing cast which has a gap in the front to allow for swelling. As it is a non-weight bearing cast, you must not walk on the affected leg. You will be given a walking aid (crutches or a frame) to take home, and an appointment to come back to Fracture Clinic several days later to check swelling and apply POP cast.

POP (Plaster of Paris)

In fracture clinic, if your swelling has reduced sufficiently, the technician will remove your Backslab and apply a cast known as a POP. Once applied, the doctor will inform you how much weight you can put through the cast. This may remain as non-weight bearing.

Aircast boot

Depending on the location and type of your fracture you may be put into an Aircast boot. These are usually used for fibula fractures. Some will require a fracture clinic follow up, others will not. You will be informed of this decision prior to your discharge home. If using an Aircast boot, the doctor will inform you how much weight you can put through the boot.

Exercises

While you are in a cast you will not be able to exercise your ankle joint, however you should try the following exercises to help with blood circulation, reduce joint stiffness and muscle weakness:

Squeeze buttock muscles.

Bend knee up towards your chest.

Straighten knee and squeeze thigh muscle.

Wiggle toes as much as the cast will allow.

Daily activities

You may find certain activities difficult due to pain, or reduced mobility. The Occupational Therapist (OT) can support you by assessing you for equipment to make tasks easier. Equipment needed can be provided on discharge from hospital.

Your home circumstances will be considered, and the OT will work with you to address any difficulties you may have e.g. you may have to consider having a bed downstairs if you are unable to manage the stairs, or you may need a commode downstairs if you do not have downstairs toilet.

You may struggle with other activities e.g. washing and dressing, the OT will also assess if any support is needed.

You may be issued a walking aid (crutches or frame) and be shown how to use these by the Physiotherapist.

You will not be allowed to drive with a cast or boot on. Contact the DVLA or visit the DVLA website for more information.

What to look out for

Fractures and lower limb plaster casts are associated with a small risk of Deep Vein Thrombosis (DVT). A DVT is a blood clot that forms within a deep leg vein, blocking normal blood flow. The risk is very low; however you should look out for symptoms including throbbing pain, calf tenderness, warm/red skin and swelling in the whole leg compared to unaffected leg.

Additionally, fractures and lower limb plaster casts are associated with a small risk of Compartment Syndrome. Compartment Syndrome is an increase in pressure inside a muscle, which restricts blood flow and causes pain. If it happens suddenly, it can be serious and needs treatment as soon as possible. The risk is very low; however you should look out for symptoms including burning pain in the muscle, numbness, weakness or pins and needles.

When to Seek Immediate Medical Attention:

Increasing pain that is not controlled by your prescribed pain relief.

Pins and needles or numbness in the foot or toes.

Skin around the foot or ankle turning pale, cold, or blue.

Sudden increase in swelling or tightness in the cast or boot.

Fever or signs of infection (especially after surgery or if you have wounds or pins).

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